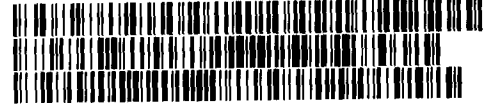


	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Tumor		☒
Case is (Circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials: <i>hw</i>	Date Reviewed: <i>9/14/11</i>	

UUID: CDD7A4EF-9901-4085-B897-975AE5E165D8
TCGA-ET-A39T-01A-PR Redacted



SPECIMEN TAKEN:

FINAL DIAGNOSIS -----

1) THYROID (THYROIDECTOMY):

SPECIMEN TYPE:

Total thyroidectomy

100-0-3

Carcinoma, papillary, follicular variant 8340/3

Site: thyroid, nrs C73.9 hw 9/14/11

TUMOR SITE:

Right lobe

HISTOLOGIC TYPE:

Papillary carcinoma, follicular variant, see note

TUMOR SIZE:

Greatest dimension 6 cm

FOCALITY:

Unifocal

LYMPH NODES:

All lymph nodes are negative for tumor (includes parts 1 and 3).

EXTENT OF INVASION

PRIMARY TUMOR:

Tumor >4 cm, limited to thyroid or with minimal extrathyroid extension

REGIONAL LYMPH NODES:

No regional lymph node metastasis

DISTANT METASTASIS:

Cannot be assessed

MARGINS:

Margins are uninvolved

VENOUS/LYMPHATIC INVASION:

Absent

ADDITIONAL PATHOLOGIC FINDINGS:

2) LYMPH NODE, PRE-TRACHEAL (LYMPHADENECTOMY): TISSUE
INSUFFICIENT
FOR DIAGNOSIS.

3) NODAL TISSUE, RIGHT CENTRAL NECK (LYMPHADENECTOMY): TWO (2) BENIGN LYMPH NODES.

Note: The neoplasm shows varying degrees of atypia. While much of the neoplasm is composed of well-differentiated cells, focally, the presence of overlapping nuclei, nuclear pseudoinclusions, nuclear grooves, and nuclear atypia reach the diagnostic level of a follicular variant of papillary carcinoma. This case was shown at our